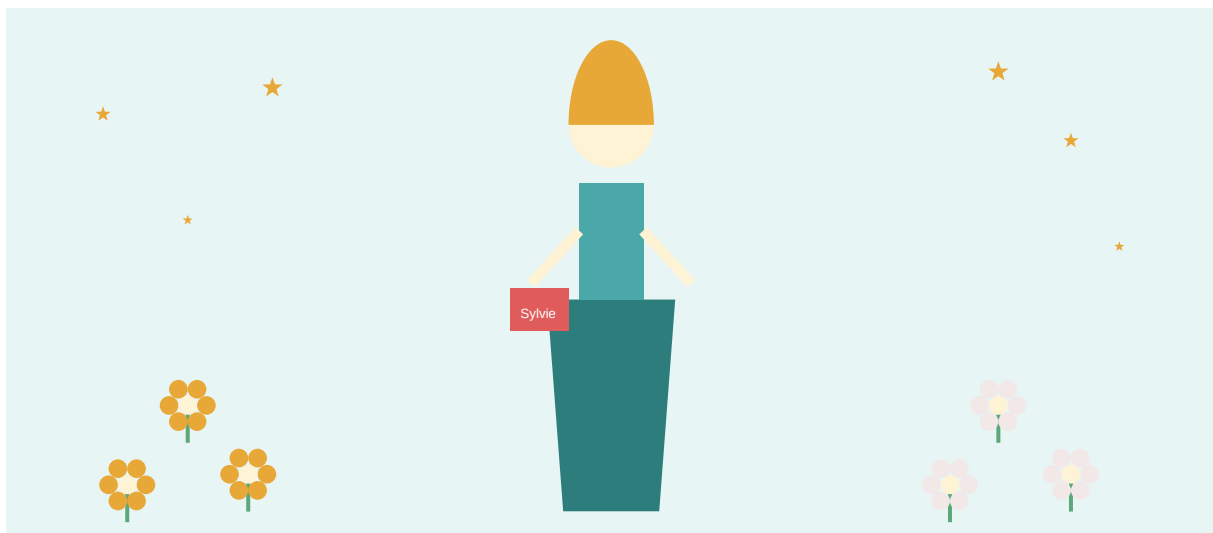


A Family Guide to Her Neurodevelopmental Profile

Kristy & Josh Parris • April 2026



This report brings together Sylvie's OT handover letter, NDIS documentation, allied health observations, and a research-based analysis of her profile. It is written for you — her parents — as a clear, compassionate roadmap for understanding and supporting her.

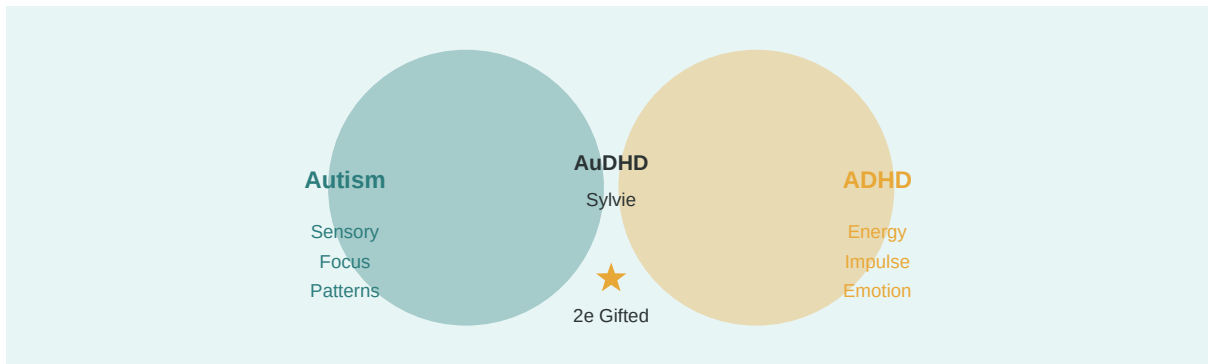
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■ 1. Primary Neurodevelopmental Profile

Sylvie's profile is best understood not as a list of problems, but as a unique operating system — one that has real strengths and genuine needs. Three overlapping frameworks help explain her experience.



Autism Spectrum Disorder (Confirmed)

Sylvie has a formal identification of Autism, with paediatrician documentation pending. Her presentation aligns strongly: sensory processing differences, monotropic (deep, narrow) focus, challenges with flexible thinking when things feel 'wrong', and difficulties navigating neurotypical social scripts.

Key insight: Autism is Sylvie's primary neurotype — it shapes how she processes the world, what feels safe, and how she communicates when words fail.

Suspected ADHD — Highly Probable

Josh has a formal ADHD diagnosis, providing a strong genetic basis. At age 4, it is difficult to cleanly separate ADHD impulsivity from autistic dysregulation. Sylvie's high energy, aggressive outbursts when overwhelmed, and intense emotional reactivity strongly suggest a combined **AuDHD profile**.

Twice-Exceptional (2e) — Gifted and Neurodivergent

Sylvie's advanced numeracy, rich vocabulary, and imaginative play point to a twice-exceptional (2e) profile. Her intellect understands what she wants to do — her nervous system cannot always execute it under stress. This gap is the source of immense internal frustration.

"Her intellect understands what she wants to do, but her nervous system cannot execute it under stress. This asynchronous development is the engine of her meltdowns."

Secondary Friction Points

• Pragmatic Language

Strong vocabulary but struggles with conversational repair, emotional communication under stress, and social scripts. Under pressure, words collapse — behaviour takes over.

- **Sensory Processing**

What looks like defiance (wall scribbling, hygiene refusal) is primarily sensory avoidance and nervous system overload — not willful opposition.

- **Separation / Transition**

Daycare drop-off distress and night resettling are both expressions of her nervous system lacking the predictive safety needed to shift contexts.

- **Sleep Dysregulation**

AuDHD sleep disruption is neurochemical and sensory — not a behavioural choice. Co-regulation with Mum is a biological need, not a bad habit.

Recommended Assessments

Paediatric Medical Review [HIGH PRIORITY]

Finalise formal Autism documentation. Screen for hidden physical drivers of night waking and dysregulation: ferritin/iron, constipation, sleep-disordered breathing, eczema.

Formal Psychological Assessment [MEDIUM PRIORITY]

WPPSI-IV and Vineland-3 to map her 2e profile definitively before school enrolment. Essential for school funding and support documentation.

Comprehensive Sensory Profile [IMMEDIATE PRIORITY]

Distinguish which sensory inputs trigger aggression versus which organise her nervous system. Guides OT intervention and home environment design.

■ 2. Sleep: What Works and Why

The core paradigm shift: nighttime waking is a **nervous-system threat response**, not a behavioural refusal. When Sylvie wakes in the dark, her distress is genuine dysregulation — not boundary testing.



Traditional Sleep Training (Ferber etc.)

Assumes the child can self-regulate but chooses not to. For an AuDHD child, self-regulation in the dark is often biologically unavailable. Planned ignoring escalates anxiety to panic, spikes cortisol, and reinforces bed = trauma.

✗ Contraindicated for AuDHD

Attachment-Informed Approaches

Responds to proximity as biological necessity. A floor bed beside Sylvie allows immediate co-regulation before she reaches full meltdown, turning a potential 2-hour ordeal into a brief resettling.

✓ Recommended

Sensory-Informed Approaches

Heavy proprioceptive input 30 min before bed, white/brown noise, minimal tactile irritants, and a weighted blanket (if preferred and safe) lower the sensory friction of the sleep transition significantly.

✓ Recommended

Neurodiversity-Affirming Support

Drops the neurotypical expectation of uninterrupted solo sleep. Focuses on a safe, calm environment rather than forcing independence. Reduces family-wide anxiety immediately.

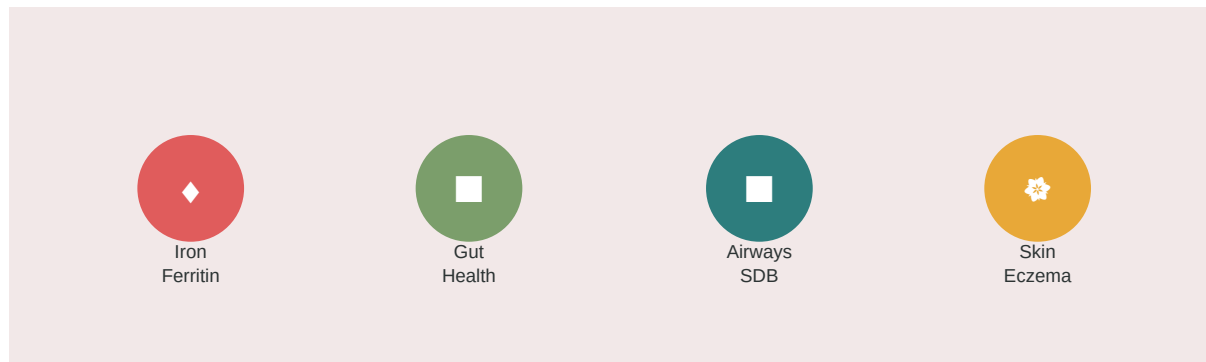
✓ Recommended

Practical synthesis:

1. Rule out physical causes first (iron, adenoids, constipation, eczema).
2. Set up a floor bed — proximity without the fight.
3. Use a visual night map: 'I wake → I hug Bear → Mum comes.'
4. Drop the sleep training guilt. Regulated sleep with support is developmentally superior to traumatic independence.

■ 3. Hidden Medical Contributors

When Sylvie's nervous system is already processing the world at a high stress baseline, any additional physiological burden — especially hidden pain — rapidly depletes her capacity to regulate. Because autistic children often have altered interoception, pain is communicated as aggression, sleep refusal, or sensory explosion rather than words.



1. Constipation & GI Discomfort

GI issues are significantly more common in autistic children. Chronic constipation causes low-grade, fluctuating pain that directly impacts mood, anxiety, and sleep via the gut-brain axis.

Watch for:

- Aggression spiking 30–60 min after eating
- Pushing stomach against hard surfaces or hiding
- Hard/pebble-like stools, or overflow smearing
- Mood crash that reliably follows meals

GP/Paediatrician action: Physical palpation; trial of Macrogol/Movicol to see if behaviour improves; abdominal X-ray if impaction suspected.

2. Iron Deficiency (Low Ferritin)

Iron drives dopamine and serotonin production. Low ferritin causes Restless Legs Syndrome in children — an uncomfortable 'creepy-crawly' feeling that makes lying still agonising and closely mimics ADHD hyperactivity.

Watch for:

- Frequent night waking, kicking or thrashing in sleep
- Needing constant leg movement or rubbing to settle
- Extreme daytime fatigue presenting as hyperactivity
- Pica (chewing non-food items)

GP/Paediatrician action: Full iron panel specifically checking ferritin. Specialist threshold: >50 µg/L (standard labs may accept as low as 15 µg/L).

3. Sleep-Disordered Breathing (SDB)

Enlarged tonsils or adenoids partially block the airway during sleep, causing micro-arousals through the night. The child spends the night in low-oxygen, high-cortisol stress — completely destroying restorative sleep.

Watch for:

- Audible mouth breathing, snoring
- Sleeping with neck hyper-extended
- Heavy night sweating, bedwetting
- Waking up instantly irritable and unrefreshed

GP/Paediatrician action: Tonsil examination; ENT referral to assess adenoids; consider polysomnography.

4. Hidden Allergies & Eczema

Autistic children frequently have co-occurring atopic conditions. Eczema itch worsens at night and — combined with sensory differences — is experienced as severe, consuming pain rather than a minor irritation.

Watch for:

- Dry patches in elbow/knee creases
- Rubbing eyes; chronic runny nose
- Scratching during sleep; sudden aggression in pollen season

GP/Paediatrician action: Full skin check; topical corticosteroids to break itch-scratch cycle; referral to paediatric allergist.

5. Neurological Pain (Headaches/Migraines)

A 4-year-old rarely says 'I have a headache' — it presents as sudden behavioural collapse. Sylvie is also at slightly elevated risk for absence seizures, which can look like spacing out or cause massive post-ictal fatigue.

Watch for:

- Head-pressing against floor or self-hitting head
- Sudden extreme light/sound sensitivity beyond her baseline
- 'Spacing out' episodes

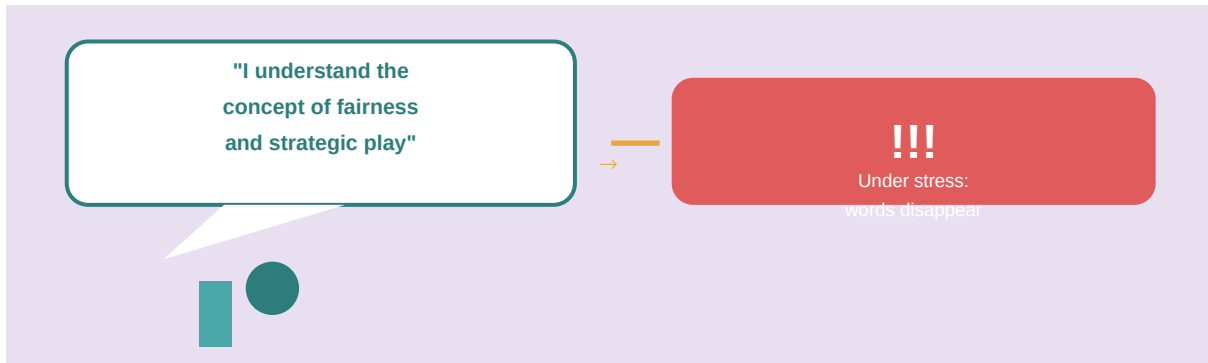
GP/Paediatrician action: Neurological exam; ear check for glue ear; EEG if spacing out is frequent.

How to approach your next medical visit:

1. Lead with physiology: *"We need to rule out iron deficiency, constipation, and sleep-disordered breathing before assuming this is purely autism/ADHD."*
2. Bring video: 30-second clips of night thrashing, post-meal aggression, or unusual sleep positions are highly persuasive.
3. Request tiered, least-invasive screening first: physical exam → blood test → specialist referral as needed.

■ 4. Pragmatic Language & the Language Mask

The most critical concept: when a 4-year-old has an advanced vocabulary, adults unconsciously assume her social and emotional language is equally advanced. In Sylvie's 2e/AuDHD profile, expressive language and pragmatic (social) language develop highly **asynchronously** — and under stress, expressive language collapses first.



The Sibling Trigger

Pragmatic language governs conversational repair — what to do when communication breaks down. Sylvie knows exactly how to articulate a demand. But when a 1-year-old doesn't follow the script, she runs out of tools. Hitting is the result of an exhausted communicative toolkit, not malice.

The Social Entry Problem

High verbal intelligence doesn't confer the invisible social scripts for joining a group, gaining attention, or greeting. She may desperately want peer connection but lack the intuitive 'entry script' — leading to hovering, sudden interrupting, or physically intense attempts to connect that others misread as aggression.

Emotional Vocabulary vs. Real-Time Interoception

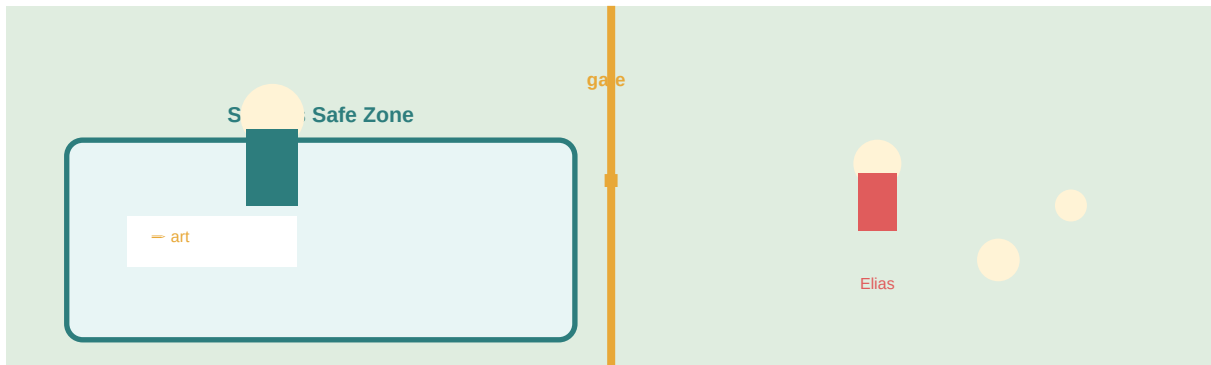
Sylvie can identify emotions in a picture book. But applying those words to her own state in the moment requires interoception and working memory — both of which go offline during dysregulation. Asking 'use your words' during a meltdown is an impossible demand.

Practical Strategies

- **Script the Pivot:** During calm moments, practice the 'Trade Script' for sibling play: 'If words don't work, give a trade or walk away.' Repeat until motor memory.
- **Teach Social Entry Explicitly:** Break down the mechanics literally: 'When we want to play, stand near them and say Can I have a turn?' Explain why: 'Saying excuse me makes their ears ready.'
- **Visuals for the Highly Verbal:** Never assume full sentences mean visuals aren't needed. A simple chart showing 'I made a mistake' → options (Breath / Try again / Ask for help) provides a lifeline when verbal reasoning fails.
- **The Post-Game Autopsy:** Review challenging moments long after they're over, using declarative language: 'I saw you trying to join those kids. It's really tricky to know how. Next time we could try holding out a toy.' Never during, always after.

■ 5. Sibling Dynamics with Elias

Sylvie's nervous system requires high predictability to feel safe. A mobile, unpredictable 1-year-old is not just an annoyance — he is a walking sensory and routine disruption. The goal is not to force them to 'play nicely' right now, but to **design an environment where both are safe**.



Proactive Environmental Design

- **The Elias-Free Sanctuary:** Guarantee a physical space — gated, high table, or room — where Sylvie's complex play cannot be destroyed. If she knows her work is protected, her baseline anxiety around Elias drops significantly.
- **Visual Ownership:** A specific coloured mat = Sylvie's space, Elias cannot come on. Parents enforce the boundary physically so she doesn't have to resort to hitting.
- **High-Value Toy Quarantine:** Toys that trigger high resource-guarding go away while Elias is roaming. Brought out only during his nap or in the gated zone.

In-the-Moment Coaching

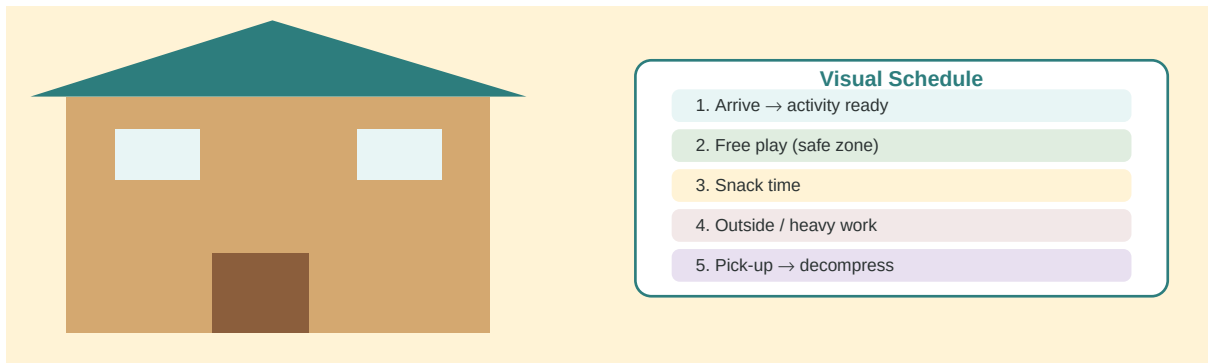
- **Silent Body Blocking:** When friction peaks, step between the children silently. 'I am keeping everyone safe' — physically prevent the hit rather than waiting to punish it.
- **The Physical Pivot:** Practice 'The Trade' (decoy toy into Elias's hands) and 'The Walk Away' during calm moments, so they become automatic under stress.
- **Avoid Shaming the Meltdown:** 'Your body is moving too fast, I need to move Elias over here.' Shame increases internal anxiety and makes the next outburst more likely.

Building the Relationship

- **Action-Based Repair:** After adrenaline fully clears (30+ min), guide a gentle action: 'Elias was crying. Let's check if he needs his blanket.' Forced apologies build resentment, not empathy.
- **Parallel Play & Shared Heavy Work:** A sandpit with a physical divider; a dance party; pushing a heavy laundry basket together. Same regulating input, protected zones.

■ 6. Preschool Environment & Inclusion

Typical daycare environments are structurally at odds with an AuDHD nervous system — acoustically bright, highly unpredictable, packed with rapid transitions, and reliant on group compliance. A single day can deplete all of Sylvie's regulatory reserves.



Drop-Off & Separation

- Have a specific high-interest activity (numeracy, building) ready in a quiet corner before arrival.
- Use a visual goodbye bridge showing: Mum leaves → sequence of the day → Mum returns.
- Quiet, low-demand handoff: 'Good morning. The magnet tiles are ready in the corner.' — no big greetings.

Protecting Focus & Play

- Ensure a physical safe zone (wall table, taped area) where her complex projects are protected from impulsive peers.
- If she is deeply engaged solo, protect that focus — her brain is regulating. Observe, don't intervene.

Transitions & Demands

■ Declarative directions: Instead of 'Pack away the blocks,' use 'The block time is finished, we need the floor clear for lunch.'

■ Pair the 5-minute verbal warning with a visual timer placed near her — no eye contact required.

Social & Emotional Support

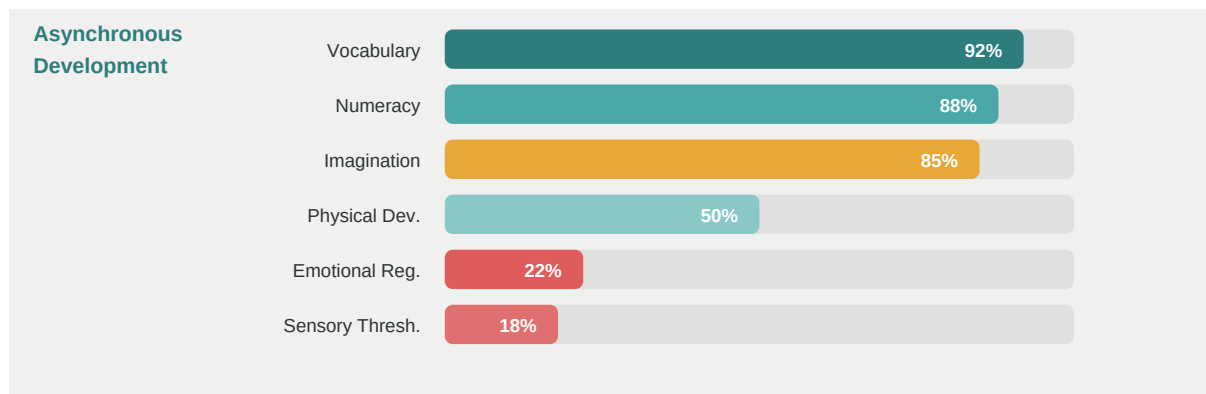
■ During resource conflict: coach the physical pivot ('I'll keep the toy safe. Let's find a spot.') — don't demand words.

■ Designated quiet space she can access any time without asking permission: tent, beanbag, outdoor nook.

■ When agitated: offer heavy work (carry a book basket, push a trolley, wipe tables) rather than time-out.

■ 7. Twice-Exceptional (2e) Profile

The defining feature: **asynchronous development**. Sylvie may have the vocabulary of a 7-year-old, the physical development of a 4-year-old, and the emotional regulation capacity of a 2-year-old. This extreme internal unevenness is exhausting for her — and highly confusing for caregivers.



Advanced Vocabulary vs. Emotional Collapse

Her expressive language is so advanced that adults expect her emotional language to match. But under stress, her brain bypasses language entirely. She knows the word 'frustrated' — she cannot access it when her tower falls.

Patterning & Numeracy vs. Cognitive Inflexibility

Her exceptional memory and systemic logic mean she knows exactly how things should be. When Elias disrupts a carefully constructed pattern, it's not just annoying — it's a breakdown of logical order her autistic neurology cannot tolerate.

Intense Imagination vs. Emotional Overwhelm

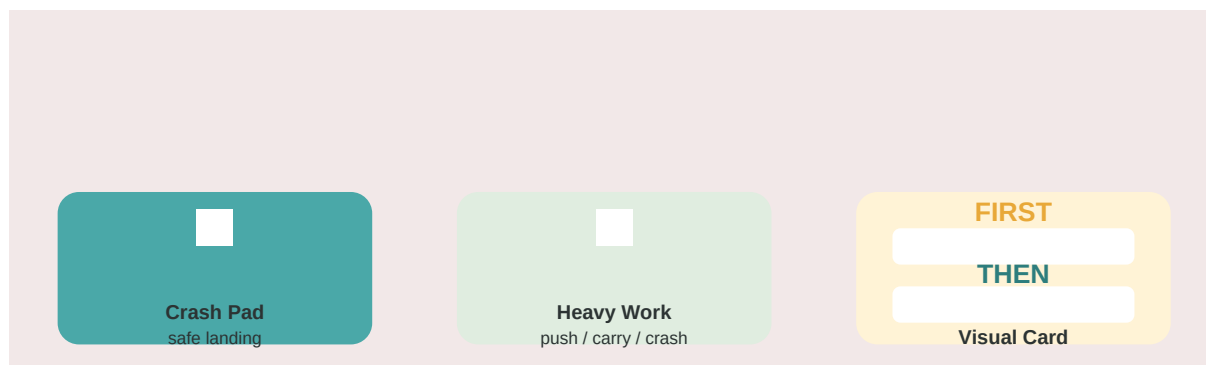
A slight mistake in a drawing isn't a physical error — her advanced cognition magnifies it into catastrophic failure. Her brain is processing more data than a neurotypical peer, leading to faster sensory exhaustion.

Key Strategies for a 2e Profile

- **Scaffold the Deficit, Feed the Strength:** Use her cognitive strengths to bypass vulnerabilities.
 - Example: use her numbered sequencing for hair washing — let her count steps while the sensory input is happening. Don't make access to intellectual interests conditional on regulation.
- **Visuals Even for Verbal Children:** Visuals reduce cognitive load, not language comprehension. Even complex sentences don't mean transition visuals are unnecessary — they save her executive energy for the emotional work itself.
- **The 'Two Brains' Concept:** Tell her: 'You have a Thinking Brain that is incredibly fast, and a Feeling Brain that sometimes gets too loud and needs help.' This removes shame from dysregulation and frames it as a mechanical problem to solve together.
- **Protect the Flow State:** Guaranteed uninterrupted focus time every day is not just play — for a 2e child it is essential neurological recovery. Guard it like a medical necessity.

■ ■ 8. Safety Planning at Home

When Sylvie begins destroying property or hurting others, her language processing and impulse control have completely shut down. The burden of safety must shift from her internal control to the **external environment**.



Environmental Modification ('Yes Space')

- Rotate toys — only a few highly durable items accessible during high-risk times.
- Install a designated 'legal scribbling' surface (perspex sheet or chalkboard panel on the fence). Give the urge an outlet.
- Bolt all bookshelves and tall furniture to walls.

Low-Tech Regulation Tools

- Crash pad or durable mattress in a central spot — redirect energy physically: 'If your body needs to crash, crash here.'
- Remove hard objects from reach; stock heavy-but-soft items (rolled socks, soft beanbags) for throwing.
- Silicone chew necklace available for jaw tension and self-directed behaviour.

Crisis De-escalation

- Silent body blocking — step in smoothly as a physical barrier, no eye contact, no lecture.
- Minimise language to one calm, repeated phrase maximum: 'I am keeping you safe.'
- Adults: lower to the floor, slow your breathing. Her nervous system will co-regulate to yours.

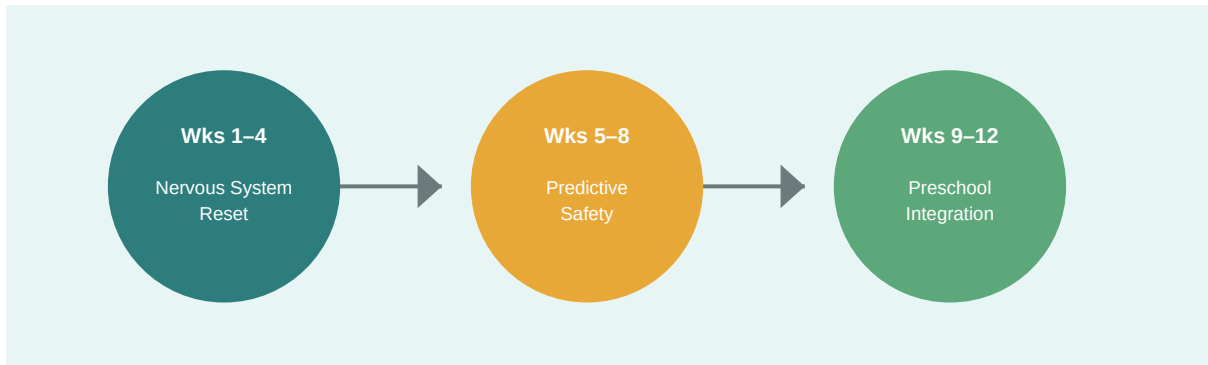
Tracking to Predict Risk

- Track sleep quality (night wakings), bowel movements, food intake — the 'Body Budget.'
- Log sensory load: daycare day? Noisier environment? How many forced transitions?
- Use this data to preemptively reduce demands on high-load days rather than being surprised.

The most powerful safety tool is a secure attachment. Ensure multiple points each day where Sylvie experiences physical connection and shared joy — completely disconnected from therapy goals, behaviour management, or sibling logistics.

■ 9. The 12-Week Parent-Mediated Plan

When family bandwidth is low and the household includes a toddler, traditional 'therapy homework' is unsustainable. This model focuses on the **Minimum Effective Dose** — the smallest set of daily and weekly actions that produce the highest regulatory return. The key principle: changing the *environment and interaction style* consistently outperforms trying to change the child's skills through repetition.



Drop These Immediately (Low Yield)

- Verbal de-escalation during a meltdown
- Forced apologies after sibling aggression
- Structured 'therapy homework' at home when Sylvie is already exhausted
- Multiple clinic visits per week dragging both kids into waiting rooms

Focus Exclusively Here (High Yield)

- Environmental architecture: physically separating Sylvie's play from Elias's reach
- Declarative language: observations instead of commands
- Visual predictive safety: pictures to map daily rhythm and transitions
- Co-regulation: sitting silently beside a dysregulated child rather than fixing

Weeks 1–4: Nervous System Reset

- Establish the impregnable sibling sanctuary daily.
- Audit all daily demands — drop non-essential commands; shift to declarative language.
- Consolidate allied health to a Key Worker model (OT leads, coordinates SP + preschool).

Weeks 5–8: Predictive Safety & The Pivot

- Implement extreme low-fidelity visuals: just a whiteboard with two pictures — FIRST / THEN.
- Silent body blocking becomes the default sibling conflict response.
- Practice the 'Trade Script' as a game during calm weekend moments.

Weeks 9–12: Preschool Integration & Recovery

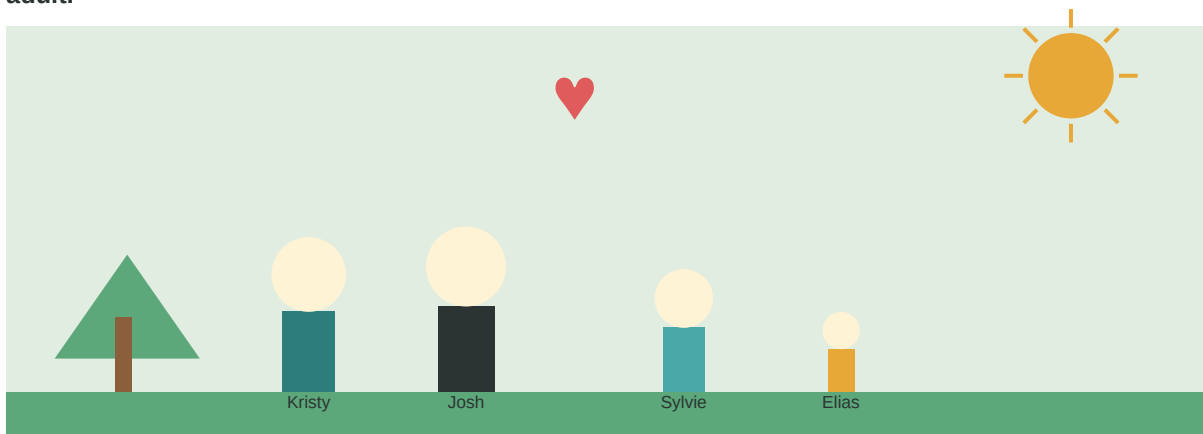
- Visual map for drop-off sequence; eliminate all verbal chatter during the car ride.
- Designate the afternoon after preschool as a 'decompression chamber': no demands, heavy snack, dim lights, quiet tent access.
- Weekly post-game autopsy: declarative review of one social challenge from preschool.

Weekly Operating Rhythm

- **Sunday Night Sync (15 min):** Identify the highest-stress day ahead. Pre-plan the easy dinner. Coordinate which parent manages Elias so the other protects Sylvie's decompression time.
- **The 'One Thing' Rule:** Each day, consciously practise only ONE intervention strategy. Monday: declarative language. Tuesday: first/then visual. Never run all strategies at once.
- **Guaranteed Micro-Respite:** Each parent protects a 30-minute block of complete off-duty time every single day. This is a clinical necessity, not a luxury.

♥ 10. Family Wellbeing & a Rule of Life

Sylvie's outcomes are inextricably linked to your nervous systems. Therapy for the child cannot out-compete severe depletion in the family unit. The most potent intervention for an AuDHD child is a **deeply regulated adult**.



- **Parental Nervous-System Regulation:** Sylvie is highly attuned to your physiological state via neuroception. If either parent is chronically running on cortisol, her system registers ambient threat — lowering her meltdown threshold. Protecting your sleep is a clinical intervention.
- **Divide and Conquer (Not Whole-Family):** Research shows successful AuDHD families frequently use structured division of labour. One parent takes Sylvie for deep-focus low-demand time; the other takes Elias for high-energy play. Both parents get to experience competence, not constant crisis.
- **Rhythm over Rigid Routine:** Autistic brains need predictability, but rigid timetables shatter under family stress. A rhythm — same sequence regardless of clock — provides safety without a pass/fail dynamic. 'After breakfast, we always go outside' rather than '8:30 AM outside.'
- **Faith & Meaning-Making:** Families grounded in shared spiritual practices show significantly higher resilience against caregiver burnout. Simple predictable rituals — a worship song to end the day, a memory verse during transitions — serve double duty: sensory and emotional regulation for Sylvie, grounding for you.

A Proposed Weekly Rule of Life

Daily Anchors	• Morning: 10 min outdoor light before the rush (sets circadian rhythm for AuDHD sleep). • 4–5 PM Zone Defence: parents divide — Elias outside/kitchen, Sylvie in sanctuary (protected, screen-free). • 5 PM strategic screen-time drop: low-stimulation show while dinner is prepared. • Evening Liturgy: predictable 15-min sequence — bath/wipe-down → Bible verse/prayer → white noise.
Saturday	• Morning: one parent takes both kids out for 90 min. Other parent has total silence at home. • Afternoon: swap. Both parents get guaranteed off-duty time every weekend.

Sunday

- Low-demand Sabbath: protect from therapy homework, major chores, extensive socialising.
- Family rhythm: story + song + slow walk. If church is too dysregulating right now, a home rhythm is valid.
- Evening Sync (15 min after kids are down): identify the hardest day ahead, decide dinner.

A closing word:

Sylvie is an extremely kind, fun, and active young person. Her intensity, her love of patterns and numbers, her fierce imagination, and her deep need for fairness — these are not problems to be fixed. They are the shape of who she is. With understanding, the right environment, and a family who knows how to read her, she will build the capacities she needs — in her own time, at her own pace.

"She is extremely kind, fun and active. With continued support, she will build her capacities to ensure her long-term daily participation."

— Ruby Chandran, OT, Enhancing Independence, April 2026



Prepared for Kristy & Josh Parris • April 2026 • Sylvie Margaret Parris (DOB 26 Feb 2022)
Sources: OT Handover Letter (Ruby Chandran, Enhancing Independence, April 2026); NDIS Plan; parent observations; peer-reviewed research on AuDHD, 2e profiles, sensory processing, and neurodiversity-affirming intervention.